

REVENUE CYCLE
KNOWLEDGE BANK

Revenue Cycle Process Flowchart

From patient access to final account resolution

Included

One-page end-to-end flowchart, exception loops, functional swimlane, stage controls, ownership matrix, EDI transaction map, and LMS activity ideas.

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End-to-End Revenue Cycle Flow

The revenue cycle begins before care is delivered and ends only when payer and patient balances are correctly resolved. Errors created upstream often reappear as rejections, denials, underpayments, delayed cash, or incorrect patient responsibility.



FINAL CONTROL: Reconcile → close valid balances → trend root causes → correct upstream process → monitor KPIs

Core exception loop

Rejected claim → correct supported data → resubmit. Denied or underpaid claim → validate remit and policy → corrected claim, reconsideration, appeal, contract dispute, or adjustment as appropriate. Never rebill blindly.

Functional Swimlane and Key Handoffs

Swimlane	Primary responsibility	Critical output / handoff
Patient / Guarantor	Provides demographics, insurance, consents, payment, and requested information.	Accurate information, informed expectations, timely response.
Patient Access	Schedules, registers, verifies eligibility and benefits, checks network, authorization, estimate, and collections.	Complete encounter record handed to clinical and billing teams.
Clinical Team	Documents the encounter, orders, services, medical rationale, and results.	Complete and signed documentation supports coding and billing.
Health Information / Coding	Reviews documentation and assigns supported diagnosis, procedure, modifier, and claim-related coding.	Coded encounter passes to charge and billing workflow.
Charge Integrity / Billing	Captures charges, creates the claim, applies edits, and submits to the correct payer.	Accepted claim with traceable submission evidence.
Clearinghouse / EDI	Validates transaction structure, routes claims, and returns acknowledgments.	999 and 277CA or equivalent acceptance/rejection information.
Payer	Applies eligibility, coverage, coding, medical policy, contract, and benefit rules.	Payment, adjustment, denial, or request for information.
Payment Posting	Posts EFT/check, ERA/remit, adjustments, patient liability, and provider-level transactions.	Accurate account balance and follow-up status.
AR / Denials / Appeals	Researches unpaid, denied, or underpaid claims and selects the correct resolution path.	Payment, corrected adjudication, valid adjustment, escalation, or appeal decision.
Patient Financial Services	Issues statements, answers questions, applies assistance/payment arrangements, and resolves valid patient balances.	Final patient balance resolved under policy.
Leadership / Analytics / Quality	Monitors KPIs, trends defects, confirms ownership, and drives prevention.	Upstream corrective action and sustained process improvement.

Handoff principle

Every handoff should transfer both the work item and the evidence needed to complete the next step. Missing authorization proof, incomplete notes, claim acceptance evidence, or unclear account documentation creates avoidable rework.

Decision and Exception Flow

Decision		Branch	Required action
Was coverage active and correct?	No		Correct insurance, update COB, identify correct payer, or route valid patient/self-pay workflow.
Was authorization required and obtained?	No		Check exception or retro-review rules, collect evidence, and route for payer review or approved financial workflow.
Is documentation complete and coding supported?	No		Return to authorized clinical, coding, or charge owner. Do not infer or add unsupported information.
Was the claim accepted?	No		Read rejection acknowledgement, correct the supported defect, and resubmit before the deadline.
Was the claim paid as expected?	No		Review remit, contract, benefits, claim status, and policy; identify denial, underpayment, or posting issue.
Is the remaining patient balance valid?	No		Correct posting, COB, payer responsibility, contractual adjustment, or notices before billing.
Is the account fully and correctly resolved?	Yes		Close, retain required evidence, and feed root-cause information into prevention and reporting.

Stage Controls and Failure Prevention

Stage	Control checkpoint	Typical failure	Useful KPI
Scheduling & registration	Correct patient, subscriber, provider, service, and contact data.	Duplicate record, invalid member ID, wrong payer.	Registration accuracy; preregistration rate.
Eligibility & benefits	Verify active coverage for DOS, network, benefits, accumulators, and COB.	Inactive coverage, benefit exclusion, incorrect payer order.	Verification rate; eligibility-denial rate.
Authorization & referral	Match service, code or category, units, dates, provider, and site.	Missing or invalid authorization, exceeded units.	Authorization rate; auth-denial rate.
Clinical documentation	Complete, timely, signed, and internally consistent record.	Medical necessity denial, coding delay, compliance risk.	Documentation completion; DNFB.
Charge capture & coding	Reconcile documented services to charges and supported coding.	Missing revenue, invalid code, bundling or modifier defect.	Charge lag; late charge rate; coding accuracy.
Claim creation & submission	Validate payer routing, identifiers, claim fields, edits, and attachments.	Rejection, wrong payer, incomplete claim.	Clean claim rate; rejection rate.
Adjudication & remittance	Review group code, CARC, RARC, line details, allowed amount, and payer message.	Incorrect posting, overlooked denial, wrong liability.	Denial rate; first-pass resolution.
Payment posting	Reconcile EFT/check to ERA/remit and post all lines and adjustments.	Unapplied cash, incorrect balance, duplicate follow-up.	Posting lag; unapplied cash.
AR & denial resolution	Use status, history, evidence, deadlines, and payer-specific resolution route.	Repeated rebilling, missed appeal, untimely action.	Days in A/R; aged A/R; overturn rate.
Patient billing & closure	Validate patient liability, secondary billing, notices, assistance, and final posting.	Incorrect statements, complaints, credit balances.	Patient collection rate; credit balance rate.

Educational process map | Adapt to payer, provider, and organizational requirements

Stage	Control checkpoint	Typical failure	Useful KPI
Analytics & governance	Use governed definitions, root-cause ownership, trend review, and action tracking.	Conflicting metrics, repeated defects, weak accountability.	KPI trend, avoidable denial rate, action closure.

Electronic Transaction Map

Why transactions matter

HIPAA Administrative Simplification establishes standards for electronic administrative healthcare transactions. In practice, these transactions connect eligibility, claims, status, and remittance workflows across providers, clearinghouses, and payers.

Transaction	Purpose	RCM point	Operational reminder
270 / 271	Eligibility and benefit inquiry / response	Before or around service	Coverage and benefits information; point-in-time response does not guarantee payment.
278	Healthcare services review	Before specified services	Electronic authorization or referral workflow where adopted and applicable.
837P / 837I / 837D	Professional / institutional / dental claim	After coding and claim creation	Transmits claim data to a payer or clearinghouse.
999	Functional acknowledgment	Immediately after EDI receipt	Indicates structural acceptance or rejection of the transaction set.
277CA	Claim acknowledgment	After front-end claim edits	Reports whether individual claims were accepted or rejected before adjudication.
276 / 277	Claim-status inquiry / response	During follow-up	Returns status information for a submitted claim.
835 / ERA	Claim payment and remittance advice	After adjudication	Communicates payment, adjustments, denials, and patient responsibility.
EFT	Electronic funds transfer	At payment	Moves funds to the provider; the ERA explains the related claim adjudication.

Official References

- **CMS Administrative Simplification:** <https://www.cms.gov/priorities/key-initiatives/burden-reduction/administrative-simplification>
- **CMS Electronic Health Care Claims:** <https://www.cms.gov/medicare/coding-billing/electronic-billing/electronic-healthcare-claims>
- **CMS Health Care Payment and Remittance Advice:** <https://www.cms.gov/medicare/coding-billing/electronic-billing/health-care-payment-remittance-advice>
- **CMS Payment, Remittance Advice, and EFT:** <https://www.cms.gov/priorities/key-initiatives/burden-reduction/administrative-simplification/transactions/health-care-payment-remittance-advice-electronic-funds-transfer>

Educational disclaimer

This process map is a general educational model. Actual workflows differ by payer, provider type, specialty, state, plan, contract, EHR or billing system, and organizational policy. The map is not legal, clinical, coding, accounting, contractual, compliance, or payer-specific advice. Current official requirements and approved procedures govern.

Key learning message

Revenue cycle management is not a straight line. It is a controlled cycle of data capture, clinical documentation, compliant billing, payer adjudication, payment reconciliation, exception resolution, patient communication, and continuous improvement.